



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مروه محمد احمد حميده
المحافظة : القاهرة
الحى / المدينة : النزهه
العنوان : النزهه
رقم البطاقة : 28508282300442 رقم التليفون : 01000470604
العمر (تاريخ الميلاد) : 28-08-1985 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مروه محمد احمد حميده National ID: 28508282300442	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

مروه
محمد احمد حميده
١٧ ميدان بقطر
النزهة - القاهرة

١٩٨٥/٠٨/٢٨ ٢٨٥ ٠٨ ٢٨ ٢٣ ٠٠٤٤٢

JA4064258

٢٠٢١/٠٨ ٢٨٥ ٠٨ ٢٨ ٢٣ ٠٠٤٤٢

بدون عمل
أنثى
مسلمة
متزوجة
محمد هديه صباير حسن
البطاقة سارية حتى ٢٠٢٨/٠٨/٢٧

١

Patient information

Patient name: Marwa Mohamed Ahmed

Date: 08-08-2023

Ref. By : Prof. Dr.

C.T. SCAN OF THE CHEST , ABDOMEN AND PELVIS

Techniques

Helical C.T. scanning of the chest , abdomen and pelvis was obtained on multidetector CT unit after IV contrast enhancement.

Chest

- The lung parenchyma appears well aerated with no obvious alveolar or interstitial infiltration. The visualized main, lobar and segmental bronchi appear patent.
- The visualized mediastinal and hilar vascular structures are within normal limits with no obvious significant lymph node enlargement or mass lesion.
- The pleural sac and bony chest wall are within normal limits.
- Evidence of multiple enlarged right axillary lymph nodes are seen , the largest measuring about 33 x 22 mm .

Abdomen

- The visualized liver is average in size showing regular outline with no obvious focal mass lesion or dilated biliary radicle. The spleen is not enlarged with no obvious abnormal parenchymal attenuation or perisplenic collection.
- The opacified stomach and duodenal loop are regularly opacified with no obvious gross ulcerative changes, spasticity or mass lesion. The small and large bowel loops appear intact with no obvious abnormal mural thickening, stenotic segment, diverticular changes or mass lesion. There is no obvious peritoneal adhesive changes or nodulations.
- The retroperitoneal structures including the kidneys, adrenal glands, pancreas and great vessels are within normal limits. There is no obvious significant lymph node enlargement or ascitic collection.
- The examined pelvis revealed regular filling of the urinary bladder with no obvious mural thickening or diverticular changes.

Name: مروه محمد احمد وفديه		
MR. : 4884		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 40-year-old female with a history of invasive ductal carcinoma, Grade II, T2 N2, estrogen receptor 8/8, progesterone receptor 8/8, HER2 negative. She underwent upfront breast conserving surgery followed by adjuvant chemotherapy and radiation therapy. Post-treatment, she has been receiving an aromatase inhibitor (Femara) and Zoladex since May 2024, with ongoing therapy for the current visit. During the visit on 21 May 2026, the patient continued her aromatase inhibitor at 2.5 mg daily and Zoladex, with supportive treatment. The plan was to maintain this regimen for an additional three months, with a pharmacy refill scheduled accordingly. The most recent note dated 15 May 2054 (likely a typographical error) indicates that the patient is again on Femara and Zoladex, receiving supportive therapy for the next three months, with the next clinic visit scheduled after that period.		
تحريراً في ,		
	التاريخ :	21-09-2026
	الوقت :	
	Reham Ahmed Fathy Ibrahim	الطبيب المعالج :
يعتمد ,		

د. أحمد كمال الدري
د. محمد أبو الهدى درويش
د. عمر حسين عمر
د. وحيد حسين طنطاوي
د. ياسر عبد العظيم عباس
د. أحمد محمد منيب
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

NAME: MARWA MOHAMED AHMED	PATIENT ID: 944556
PROF. DR. AHMED GAMAL AL DIN OTHMAN	STUDY DATE: 14/08/2023
Acc No: 1247161	

BILATERAL 3D DIGITAL BREAST CONVENTIONAL & POSTCONTRAST (CEDM) MAMMOGRAPHY WITH HIGH RESOLUTION ULTRASOUND, REVEALED:-

- Both breasts show normal skin and subcutaneous tissue and nipple areola complex.
- Mammography examination categorized (ACR-d).
- The right axillary tail shows spiculated radio-dense lesion showing microcalcific focus on mammography which shows avid contrast uptake on CESM films. Sonographic correlation reveals an ill-defined hypoechoic solid mass lesion measuring 22.3 X 16.1 X 18.3 mm
- The right axilla shows two large pathological rounded lymph nodes measuring 24.6 X 17.6 and 25.1 X 24.4 mm.
- The rest of the right breast shows no other solid or cystic lesions nor abnormal as of contrast uptake.
- The left breast shows no abnormal soft tissue shadows on conventional mammography nor abnormal areas of contrast uptake on CESM.
- However sonographic examination reveals a well-defined oval shaped hypoechoic lesion located at 11 O'clock position measuring 18.4 X 7.9 mm.
- Otherwise, both breasts show predominant dense fibroglandular element which shows patchy areas of contrast uptake on CESM with superimposed multiple variable sized thin wall echo free cysts ranging 3-11 mm in diameter.
- No left breast suspicious lesions could be seen.
- No suspicious left breast clusters of microcalcification.
- No left axillary pathologically lymph nodes could be detected.

IMPRESSION :-

- Right breast axillary tail (BIRADS VI) mass lesion with underlying two pathological lymph nodes.
- Left breast upper inner quadrant benign looking lesion keeping with fibroadenoma (BIRADs III) for follow up.
- Bilateral fibrocystic disease of the breast (BIRADS II).

MUCH OBLIGED

A.PROF. AHMED HASSAN SOLIMAN
K.M

Patient Name	: الأستاذ / مروه محمد احمد	Request No.	: 13003193
Patient No.	: 28508282300442	Request Date	: 26/07/2023
Path No.	: 2368/23	Verification Date	: 08/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 25/7/2023, 2:30 pm.

Gross Examination:

- Four grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+3) Grade III.
- * Ductal carcinoma in situ: Absent
- * Perineural invasion: Absent
- * Angioinvasion: Absent
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
- PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 30% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III .

IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 8 /8).
- Her2: Negative (score 0).

Verified by :

DR.Menna Nabil

Printing Date : 12/08/2023 09:43:49

Page 1 of 2

Printed By : Dr.Marwa mahmoud



Patient Name : الأستاذه / مروه محمد احمد :	Request No. : 13003193
Patient No. : 28508282300442	Request Date : 26/07/2023
Path No. : 2368/23	Verification Date : 03/08/2023
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 37 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 25/7/2023, 2:30 pm.

Gross Examination:

- Four grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+3) Grade III.
- * Ductal carcinoma in situ: Absent
- * Perineural invasion: Absent
- * Angioinvasion: Absent

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III .

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr.Lobna Omar

Verified by :

Dr.Heba Hasan



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

- Patient ID: 28508282300442
- Name: مروة محمد أحمد حميدة
- ID number: 28508282300442

Sex:

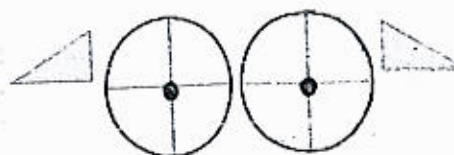
3- Medical number:
Female

83156434
Old Med :

2947/2023

Patient Age : 38 Y

MDT



Family history

Phone

01000470604

BMI

43

REASON OF MDT NEW

marital status

NRS Score

2

GROUP

B

Date of MDT 2023-09-02

Co morbidity

DM ON ORAL TTT Menopausal status

PREMENOPAUSAL

Clinical data

Breast Imaging :

Mets Workup :
CTCAP-->FREE

BX:
IDC
GII

ER:
8/8

PR:
8/8

Her 2 Neu :
0

SISH :

Ki67 :
30

T
2

N
2

M
0

Sonomammography

RT AXILLAY TAIL MASS LESION WITH UNDERLYING PATHOLOGICAL LNS
LT BREAST UPPER INNER QUADRANT BENIGN LOOKING MASS KEEPING WITH FIBROADENOMA

PATIENT HISTORY

OPERATED RT BCS+AXLND ON 20/8/2023 OUTSIDE OUR HOSPITAL
PATHOLOGY-->FREE MARGINS LN 5/17,,SLIDE REVISION REQUESTED
COMPLETED HER FAMILY

MDT OUTCOME & PLAN

FOR: ADJUVANT 4C- 4 TAXOTERE + ZOLADEX THEN PORT D OLADEX/AI + PROLIA/6MNS FOR 3 YS
IUCD INSERTION

SIGNATURE

MEDICAL ONCOLOGY

د/ منير القزوي

RADIOLOGY

د/ منير القزوي

SURGERY

د/ محمد جمال

PATHOLOGY

Psychiatric

Nutrition

شراء حريمي

Dr. Ragia Fahmy
Prof of Pathology
Ain- Shams University

Name: Marwa Mohamed Ahmed

Ref. By: PROF.DR. Ahmed Gamal

Nature of Specimen: RT Conservative breast surgery axillary tail mass and axillary clearance

Clinical Diagnosis:

PATHOLOGY REPORT

GROSS:

Specimen received is a RT conservative breast surgery (wide local excision from an axillary tail mass) measuring 9x5x3cm. The cut section shows a firm grey white mass measuring 2x2cm. No other multifocal tumors. The margins are oriented and are grossly free with the least margin 1.1cm. An axillary clearance was performed from which (17) lymph nodes were trimmed. The largest is 5x3cm. Representative sections are submitted and sectioned

MICRO:

Sections examined reveal infiltrating duct carcinoma Grade -2. The cells show moderate pleomorphism, poor attempts at tubular formations and low mitosis. There is comedo In situ element representing 5% of tumor. The stroma shows marked desmoplasia. There is solid In situ elements representing 5% of tumor. The stroma shows marked desmoplasia. There is peri tumoral lymphocytic infiltrate about 25%. There is positive lympho-vascular emboli but no perineural invasion. The rest of the breast show fibroadenosis. No necrosis. All margins are free. Sections from axillary lymph nodes show metastatic deposits in 5/17 lymph node with capsular rupture and extra nodal spread

DIAGNOSIS:

RT Conservative breast surgery and axillary clearance: infiltrating duct carcinoma grade-2, free margins and positive axillary nodes (5/17) p T2, PN2a, PMx

PROF.DR. **RAGIA FAHMY**

N.B. Enclosed are slides, report & paraffin blocks.

22/8/2023